Mood disorders

Goal of these lecture:

• To empower the students with knowledge, skills and positive attitude toward the mood disorder.

Objectives:

At the end of this presentation, participants should be able to:

• Define depression and mania.

• Mention causes of depression and mania.

• List signs and symptoms of depression and mania.

• Apply nursing management of depression and mania.

• Understanding the role of nurse in caring for patients with mood disorder.

Introduction

Mood variations are a normal part of human experience, as individuals are exposed to various life situations that influence emotions. However, when mood changes significantly interfere with emotions, cognition, behavior, and daily functioning, they may indicate a pathological condition that requires clinical attention. Mood disorders are challenging to diagnose due to the subjective nature of their symptoms. Historically referred to as affective disorders in earlier editions of the DSM (Diagnostic and Statistical Manual of Mental Disorders), mood disorders describe sustained and pervasive disturbances in emotional states.

Definition of mood disorder :

Mood disorders are psychiatric syndromes characterized by persistent disturbances in mood that affect a person's thinking, feelings, and behaviors. These disturbances last for weeks or months and significantly impair social, occupational, or daily functioning.

Type of mood disorder:

The primary categories include:

1. Depressive disorders (Depression)
2. Manic disorders (Mania)

Definition of depression :

Depression is defined as a persistent feeling of sadness, hopelessness, or emptiness accompanied by a marked loss of interest or pleasure in most daily activities. It is a clinical condition that affects emotional, cognitive, physical, and behavioral functioning.

Causes-of-depression :

1. Genetic factors: A family history of depression increases susceptibility, with onset occurring either early or later in life.
2. Biochemical factors: Neurotransmitter imbalances in the brain due to hormonal changes, inadequate physical activity, poor nutrition, or seasonal changes.
3. Environmental factors: Exposure to negative life circumstances such as poverty, abuse, violence, or social isolation.
4. Psychological factors: Maladaptive cognitive patterns and poor coping mechanisms.
5. Medication-related factors: Certain drugs may induce or exacerbate depressive symptoms (e.g., corticosteroids, beta-blockers).

- **Sign and symptoms of depression:**
- **Depression manifests in multiple domains:**
- **Emotional symptoms:**
- Persistent sadness or depressed mood
- Feelings of worthlessness or hopelessness
- Excessive or inappropriate guilt
- **Physical symptoms:**
- Fatigue or loss of energy
- Significant weight loss or gain unrelated to dieting
- Sleep disturbances (insomnia or hypersomnia)
- Psychomotor agitation or retardation
- **Cognitive symptoms:**
- Impaired concentration or decision-making
- Recurrent thoughts of death or suicidal ideation (with or without a plan)
- **Behavioral symptoms:**
- Irritability
- Withdrawal from social and pleasurable activities
- **Types of Depression:**

The American Psychiatric Association's Diagnostic Statistical Manual of Mental Disorders, Fifth Edition (DSM-5) classifies depressive disorders as the following:

1- Clinical depression (major depressive disorder):

A diagnosis of major depressive disorder means you've felt sad, low or worthless most days for at least two weeks while also having other symptoms such as sleep problems, loss of interest in activities or change in appetite. This is the most severe form of depression and one of the most common forms.

2- Persistent depressive disorder (PDD):

Persistent depressive disorder is mild or moderate depression that lasts for at least two years. The symptoms are less severe than major depressive disorder. Healthcare providers used to call PDD dysthymia.

3- Disruptive mood disregulation disorder (DMDD):

DMDD causes chronic, intense irritability and frequent anger outbursts in children. Symptoms usually begin by the age of 10.

4- Premenstrual dysphoric disorder (PMDD):

With PMDD, you have premenstrual syndrome (PMS) symptoms along with mood symptoms, such as extreme irritability, anxiety or depression. These symptoms improve within a few days after your period starts, but they can be severe enough to interfere with your life.

5- Seasonal affective disorder (seasonal depression):

This is a form of major depressive disorder that typically arises during the fall and winter and goes away during the spring and summer.

6- Postpartum depression: Major depressive episodes that begin either during pregnancy (prenatal) or within the first four weeks following delivery. According to the DSM-5, this condition is referred to as "major depressive disorder with per partum onset." Clinically, however, many experts recognize that symptoms of postpartum depression can emerge at any point during the first year after childbirth.

7- **Depressive Disorder Due to Another Medical Condition:**

Depression secondary to medical illnesses such as hypothyroidism, Parkinson's disease, cardiovascular disease, or cancer.

Management:

1. **Hospitalization:**

Who needs to be hospitalized for depression?

There are many people with depression who might benefit from a hospital stay.

Here are some examples.

- Individuals with suicidal ideation or risk of self-harm/harm to others.
- Patients unable to care for themselves due to severe functional impairment.
- Close monitoring during initiation or adjustment of pharmacological therapy.
- Administration of specialized hospital-based treatments such as Electroconvulsive Therapy (ECT).

Note: Management should include both pharmacological and non-pharmacological approaches (psychotherapy, lifestyle modification, psych education).

Ongoing follow-up and family involvement are essential for relapse prevention.

1. Pharmacotherapy:

Medications used to treat depression include

- Serotonin-norepinephrine reuptake inhibitors (SNRIs).
- Tricyclic antidepressants (TTCAs).
- Norepinephrine and dopamine reuptake inhibitors.

Antidepressants influence the overall balance of the three neurotransmitters in the brain that regulate emotion, reactions to stress, and the physical drives of sleep, appetite, and sexuality.

3. Psychotherapy (Talk Therapy)

Psychotherapy is an evidence-based treatment, often used alone in mild depression or combined with medication for moderate to severe cases.

Common types include:

1. Cognitive Behavioral Therapy (CBT): Identifies and modifies maladaptive thought patterns and behaviors; teaches coping and problem-solving skills.

2. Interpersonal Therapy (IPT): Focuses on improving interpersonal relationships, social support, and managing role transitions or grief.

3**. Psychodynamic Therapy:** Explores unconscious conflicts and relational patterns contributing to depression.

Other helpful modalities:

4**. Behavioral Activation Therapy:** Encourages engagement in positive activities to enhance mood.

5**.Family or Couples Therapy:** Useful when family dynamics exacerbate depressive symptoms.

6. **Biological (Somatic) Therapies**:

Used mainly for severe or treatment-resistant depression. These include:

Electroconvulsive Therapy (ECT): Brief electrical stimulation under anesthesia to induce a controlled seizure; highly effective for severe depression or suicidal patients.

7. **Repetitive Transcranial Magnetic Stimulation (TMS):** Non-invasive magnetic stimulation of brain regions involved in mood regulation; an option for patients not responding to medications.

These methods are considered when standard pharmacotherapy and psychotherapy are insufficient.

Nursing Role for Depressed patient:

1) **Assessment:**
   - Collect comprehensive data about the patient's physical, psychological, and social health status.
   - Assess the severity of depression using standardized tools (e.g., Hamilton or Beck Depression Scales, if available).
   - Monitor for suicidal ideation or self-harm tendencies and report immediately.
   - Evaluate sleep patterns, appetite, energy levels, mood, and daily functioning.

2) Planning:

- Develop an individualized care plan based on the patient's needs and treatment goals.
- Collaborate with the healthcare team to determine appropriate interventions (pharmacological, psychological, and biological).
- Establish short- and long-term goals, such as improving mood, increasing engagement in daily activities, and enhancing social interactions.

3) Implementation:

- Emotional support: Provide empathy and maintain a safe, nonjudgmental environment.
- Encourage expression: Help the patient express negative feelings in a healthy manner.
- Medication monitoring: Ensure adherence to prescribed antidepressants and monitor for potential side effects.
- Promote activities: Gradually encourage participation in simple daily activities, progressing as the patient improves.
- Enhance social relationships: Support communication with family and peers and encourage social engagement.
- Safety monitoring: Remove or restrict access to any means of self-harm when suicide risk is present.

4) Evaluation:

- Continuously assess improvements in mood, sleep, appetite, and overall quality of life.
- Modify the care plan based on the patient's response to interventions.
- Monitor adherence to medications and psychotherapy sessions.

5) Health Education:

- Educate the patient and family about the nature of depression and emphasize that it is a treatable condition.
- Explain the importance of medication adherence, even when symptoms improve.
- Teach warning signs of relapse and when to seek professional help.
- Promote a healthy lifestyle (regular sleep patterns, balanced nutrition, appropriate physical activity).

Definition of mania:

Mania is defined as a distinct period of abnormally and persistently elevated, expansive, or irritable mood and increased goal-directed activity or energy, lasting at least one week (or of any duration if hospitalization is required). Symptoms must cause marked impairment in social or occupational functioning or necessitate hospitalization.

Causes of mania:

The etiology of mania is multifactorial and may include:

1. Genetic predisposition or family history of mood disorders.

2. Neurochemical imbalances, particularly involving dopamine and serotonin disregulation.

3. High levels of psychosocial stress.

4. Changes in sleep patterns or significant sleep deprivation.

5. Side effects of certain medications (e.g., some antidepressants or steroids).

6. Difficult or traumatic life events.

Signs and Symptoms of Mania:

1. Inflated self-esteem or grandiosity
2. Decreased need for sleep
3. More talkative than usual or pressure to keep talking
4. 4 Flight of ideas
5. Distractibility
6. Increase in goal-directed activity (either socially, at work or school, or sexually) or psychomotor agitation
7. Excessive involvement in activities that have a high potential for painful consequences (e.g engaging in unrestrained buying sprees, sexual indiscretions, or foolish business investments).

Types of mania:

- **Hypomania (Stage I):**

A milder form of mania characterized by increased energy, decreased need for sleep, and impulsivity, but typically without psychosis or severe functional impairment. Hospitalization is usually not required.

- **Acute Mania (Stage II**)
  Marked by severe mood elevation, impulsivity, rapid speech, minimal sleep, and potential psychotic symptoms. There is significant impairment in daily functioning.

- **Delirious Mania (Stage III).**

The most severe stage, presenting with extreme disorientation, psychosis, and delirium. Hospitalization is often required to prevent harm.

Management Hospitalization:

- Ensure patient safety and communicate safety precautions with the interprofessional team to reduce risks of self-harm, suicide, or aggression.
- Provide a structured environment and ensure consistency of behavioral expectations among staff.
- Plan for quality of life, independence, and optimal recovery; refer to community resources and outpatient mental health services.

Pharmacotherapy:

- Mood stabilizers: Lithium and anticonvulsants (e.g., valproate, carbamazepine) are first-line for acute mania and maintenance therapy.
- Mood stabilizers maintenance therapy is essential in preventing recurrence of mania.
- Antipsychotics: Atypical antipsychotics (e.g., olanzapine, risperidone) are useful, especially if psychosis is present. Olanzapine is FDA-approved for acute mania.
- Benzodiazepines: Short-term use (e.g., clonazepam, lorazepam) may help with severe agitation or insomnia.
- Antidepressants: May be used for bipolar depression only in combination with a mood stabilizer, to avoid triggering mania or rapid cycling.
- Electroconvulsive therapy (ECT): Highly effective for severe or treatment-resistant mania or depression, especially when medications are failed or contraindicated.

3) Psychotherapy and Psychoeducation:

- **Cognitive-behavioral therapy (CBT):** Helps patients identify and correct distorted beliefs contributing to mood episodes.
- **Family or marital therapy:** Improves family understanding, communication, and support.
- **Interpersonal therapy:** Focuses on improving relationships and managing life stressors that affect mood stability.
- **Behavior therapy:** Targets unhealthy learned behaviors and encourages healthier coping strategies.
- **Psychoeducation:** Teaching patients and families about early warning signs, medication adherence, and lifestyle management is essential for relapse prevention.

Nursing Role for Patients with Mania

- **1. Assessment and Monitoring**
- Assess mood, thought processes, speech patterns, and energy levels regularly.
- Monitor for risk of self-harm, aggression, impulsive or risky behaviors (e.g., overspending, unsafe sexual activity).
- Evaluate sleep patterns, nutritional intake, hydration status, and weight.
- Observe for signs of psychosis (hallucinations, delusions) and escalating agitation.
- Monitor adherence to medications and assess for side effects or signs of toxicity (e.g., lithium levels).
- **2. Ensuring Patient Safety**
- Maintain a low-stimulus, structured environment to reduce overstimulation.
- Remove potentially dangerous objects; ensure supervision if patient is highly impulsive or aggressive.
- Set clear, consistent behavioral limits in a calm and non-confrontational manner.
- Implement suicide precautions if indicated.

- **3. Promoting Basic Needs**
- Provide frequent, high-calorie finger foods and fluids since patients may not sit for full meals.
- Encourage rest periods and minimize stimulating activities near bedtime.
- Assist with hygiene and grooming as needed.
- **4. Medication Management**
- Administer prescribed mood stabilizers, antipsychotics, or other medications as ordered.
- Monitor for adverse effects, educate the patient and family about the importance of medication adherence.
- Coordinate with the healthcare team regarding necessary lab tests (e.g., lithium blood levels).
- **5. Therapeutic Communication**
- Use calm, firm, and simple communication; avoid lengthy discussions or arguments.
- Provide reality orientation if patient has delusions or hallucinations; do not reinforce false beliefs.
- Show empathy while maintaining professional boundaries.
- **6. Psychoeducation**
- Educate the patient and family about the illness, medication adherence, triggers, and early warning signs of relapse.
- Provide information on stress management, healthy sleep routines, and substance abuse avoidance.
- Involve family or caregivers in discharge planning and ongoing support.
- **7. Collaboration and Continuity of Care**
- Collaborate with psychiatrists, psychologists, and social workers for comprehensive treatment planning.
- Arrange community resources, outpatient follow-up, and support groups upon discharge.

References :

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Interactive Questions Based on the Lecture Content

A) True/False Questions

1. Mood disorders are short-term mood changes that do not affect daily functioning. (…..)

2. The causes of depression include genetic, biochemical, environmental, and psychological factors. (…..)

3. Disruptive Mood Disregulation Disorder (DMDD) usually appears in children before age 10. (…..)

4. Psychotherapy is not helpful in cases of severe depression. (…..)

5. Mania is characterized by increased energy, elevated mood, and decreased need for sleep. (…..)

B) Multiple Choice Questions (MCQ)

(Choose the correct answer.)

1. Which of the following is NOT classified as a depressive disorder in DSM-5?

a) Seasonal affective disorder

b) Major depressive disorder

c) Obsessive-compulsive disorder

d) Postpartum depression

2. Which of the following is a physical symptom of depression?

a) Poor concentration

b) Fatigue or loss of energy

c) Feelings of excessive guilt

d) Social withdrawal

3. Which medication group is used to treat mania?

a) Selective serotonin reuptake inhibitors (SSRIs)

b) Mood stabilizers (e.g., lithium, anticonvulsants)

c) Beta-blockers only

d) Benztropine

4. What best characterizes hypomania?

a) Severe hallucinations

b) Mild symptoms without marked functional impairment

c) Requires hospitalization in most cases

d) Always accompanied by delusions

5. Which of the following is part of the nurse's role in caring for a depressed patient?

a) Assessing the severity of depression and monitoring the patient

b) Avoiding communication with the patient's family

c) Not providing any health education

d) Only administering medication

C) Essay Questions

(Answer briefly or in short essays.)

1. Define mood disorders and list their main types?

2. Explain four possible causes of depression?

3. List and describe the emotional, physical, cognitive, and behavioral symptoms of depression?

4. Discuss the nurse's role in caring for a patient with depression?

5. Compare the three stages of mania: hypomania, acute mania, and delirious mania?

"**Here is a link to an educational video that you can watch to benefit from it."**

https://youtu.be/BEXm4oJYkzY?si=D_LWV5Hoc5NNOfka